

Managing Breathlessness

In the 'Understanding Heart Failure' section we said that breathlessness is a common symptom of heart failure that is caused by a build up of fluid in the lungs. You may become more out of breath when doing physical activities such as walking or climbing stairs for instance. Some people also find that lying flat brings on breathlessness.

This section will help you manage and minimise the physical discomfort of breathlessness and give you advice on how to avoid feeling worse through worry or being anxious about breathing difficulties. However, if your symptoms cause you to black out, become dizzy, or become confused, it is essential that you contact the emergency services by telephoning 999.

If you have heart failure you may become breathless at any time, but there are times when this may be more common.

Ask yourself "Looking back, when am I most likely to have breathing difficulties?"

- When I am physically active?
- When I am stressed or anxious?
- When my heart failure symptoms are worse than usual?

Breathlessness during or after physical activity

Becoming slightly short of breath during physical activity and exercise is normal. Indeed, this is important in order to build your fitness (See the 'Keeping Active' section

earlier in the manual). Even athletes get breathlessness. BUT, being so breathless that you cannot catch your breath or speak during or after exercise can be uncomfortable and frightening. Although it may seem strange to do something that makes you have symptoms, building up your fitness will reduce breathlessness in the long run – so please keep going with your exercises!

- The exercises recommended in the Physical Activity section of this manual are designed for you to build your fitness gradually and safely, with a minimum of discomfort.
- Make sure you take regular rests if you find yourself getting breathless during everyday activity like walking or housework.
- Avoid rushing. Take things steadily and calmly.
- Try and slow down before you totally lose your breath. This way your breathing will return to normal quicker.
- Try to plan your activity and if necessary do smaller amounts and take short rests in between. 'Little and often' sometimes works better than long stretches of physical activity followed by long rests (and this is also a good strategy if you have joint pain that makes it difficult to exercise).
- Think about how you are breathing when you exercise. Resist the temptation to hold your breath as this will make you more short of breath. Breathe as normally as you can.

Breathlessness due to stress or 'hyperventilation'

If you are anxious, stressed, uncomfortable or in pain, this can affect your breathing. For example you may be taking short, shallow, frequent breaths and breathing from the chest rather than the stomach. You may also find yourself sighing a lot or holding your breath. All these things can lead to 'hyperventilation' – this is when you breathe very fast and find it hard to control your rate of breathing.

Symptoms of hyperventilation include:

- feeling nervous, fidgety or frustrated
- feelings of light-headedness
- tingling or numbness in the mouth
- feeling unable to catch air
- wobbly legs
- a feeling that you want to escape from something

Hyperventilation can also lead to a 'panic attack' (as described in the 'Managing Stress' section), which although not dangerous can be frightening and unpleasant.

What can I do about it?

It is important to try and maintain or get back to a normal pattern of breathing. The relaxation techniques that we have provided in the 'Managing Stress' section and on the relaxation audio can help with this. When you breathe, your stomach not your chest should rise and fall, and it is worth checking this quickly ...

Sit down and get comfortable in your chair. Put one hand on your chest and the other on your stomach. Shut your eyes and notice what is happening when you breathe in and out. If your hand on chest is rising and falling and your hand on your stomach is not moving this is wrong. The idea is to breathe with the stomach rising and falling, not the chest.

Keep practising the breathing and relaxation exercises. If you are unsure of the correct way of breathing, ask your facilitator to check how you are doing. It may feel quite unnatural at first. Some people have fallen into a habit of hyperventilating over the years so it may take some practice to correct this.

What else can I do?

Because breathing difficulties may be a symptom of stress and anxiety, taking steps to reduce your stress levels may be helpful. Please look at the 'Managing Stress' section earlier in this manual. This is designed to help you learn how to cope with stress and anxiety. If you practise the exercises regularly, this should prevent a lot of stress and anxiety from happening. The exercises also prepare you to deal with any sudden bouts of breathing difficulties or anxiety that may develop.

Breathlessness due to a flare-up in symptoms

Being more breathless than usual can be an important symptom requiring a change in your medication. Your fluid management may need to be reviewed. Use your Traffic Light Action

Plan at the back of your Progress Tracker to decide what kind of action you need to take.

It is also worth doing the following:

- check your breathing patterns (stomach or chest?)
- practise your breathing exercises from the relaxation audio and the exercise videos

Whatever is causing your breathlessness, the breathing and relaxation techniques in the 'Managing Stress' section earlier on in this manual, will help you manage this on a day-to-day basis. Please read this section if you haven't already done so.

Managing Changes in Your Symptoms: Ups and Downs

People with heart failure often manage very well for long periods of time by sticking to the right treatment and making some lifestyle changes. However, occasionally you may experience worsening of your signs and symptoms. This may be caused by a build-up of fluid, eating or drinking foods which are high in salt, or not taking medications correctly. It can also be caused by not sleeping well, or getting other illnesses such as colds or stomach bugs. You may also experience some natural variation in symptoms.

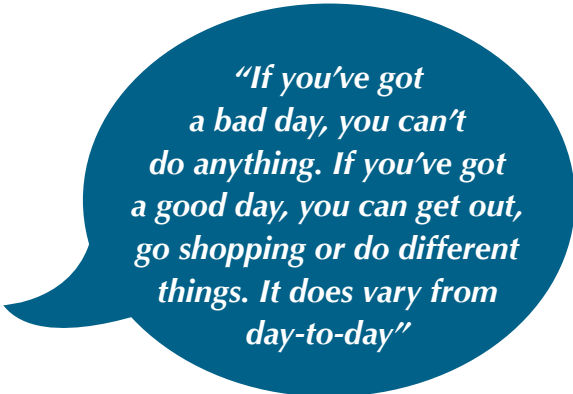
Sometimes the reason for increasing symptoms is unclear. What is clear, however, is that self care plays an important role in spotting when there is a problem and taking action to control these changes and prevent any further problems.

Having a 'Plan B' for bad days

If you find that your symptoms are changing, the Traffic Light Action Plan in your Progress Tracker gives advice on what you should do. If you are in the Green Zone, you are having a good day and that is fine. If you are in the 'Amber Zone' you may not have as much energy as usual, which may get in the way of you being able to get on with your daily activities.

Having a Plan B ready can help you make the most of these days while preventing any further problems. A Plan B should consist of things that try to conserve your energy and stop you getting over-tired. This might involve:

- taking small loads of washing to the line, rather than doing it all at once
- resting after activities and stop if you feel tired – you can come back to it later
- using a wheeled trolley for shopping rather than carrying bags
- don't plan to do harder activities right after a meal – a lot of energy goes into digesting your food, so you may have less energy after a meal
- using a high stool to sit at while doing the washing up, instead of standing
- getting other people to help you
- if you live alone, then you may be able to get help from Social Services – ask your facilitator or GP about this if you don't know how
- it is good to keep active if you can, but don't overdo it. If your symptoms are worse than usual then you may need to do your exercise at a lower level, or even have a rest day.



"If you've got a bad day, you can't do anything. If you've got a good day, you can get out, go shopping or do different things. It does vary from day-to-day"

If your daily activities are too tiring, discuss this with your facilitator or GP.

It is also important to avoid overdoing it in general – if you work too hard on one day you can be exhausted for one or two days afterwards. So avoid extreme activities and spread your exercise and other activities that require more energy through the week. It is very useful to know your limits so you don't try to do too much in a day. 'Little and often' sometimes works better than trying to get all your activity done in one go.

Figuring out what causes changes and how to manage them

Because heart failure can be quite unpredictable, it is important to keep an eye on how your signs and symptoms change over time and to try and figure out what causes these changes (i.e., what makes you feel better or worse). If you do this you may be able to avoid more serious problems which require attendance at a hospital. For instance, you might be able to improve things by getting a better night's sleep, by building up your fitness or by working with your health care team to adjust your medications.

A Progress Tracker has been provided as a separate booklet to use alongside this manual. This allows you to record how you have been feeling from week to week over a 12-week period. Using this tool may help you to learn what things seem to make your symptoms better or worse. An example of how you might use this is given on the next page...

Give it a go and see if keeping track of your progress from week to week gives you any useful ideas about how best to manage your symptoms and how to keep your spirits up.

Summary

There are many things which doctors and nurses can do to support you if you have heart failure. However, the most important person who manages the condition is **you**. There are lots of simple things which you can do to help you to lead a happy and fulfilling life and prevent the need for further treatment or hospitalisation.

This includes;

- taking your medicines as they have been prescribed
- monitoring your fluid balance
- keeping active
- managing stress and worry
- taking action if your symptoms change
- communicating well with your health care team

The Progress Tracker will help you to make these 'self-care' tasks part of your daily routine. It has been designed to help you: get the most from your medications, monitor any signs and symptoms you may have, and build up your fitness in a safe and manageable way. It will also help you to learn what things help to improve your symptoms and help you to identify any concerns that you may want to discuss with your health care team.

Example

	How have you been?	What have you done that made you feel better or worse than previous weeks?
Physically		Increased walking time and spread my shopping and washing load through the week
Mentally		Scheduled my worry time
Sleep quality		Drank too much water before going to bed

Your health care team need to know if you develop any new symptoms or if your current signs and symptoms are getting worse. Knowing what is happening to you is important, this allows your doctors or nurses to take early action to help put things right.

Finally, other people around you can often be a big help. It is worth considering what other people you live with, or friends or family who live elsewhere can do to help with any of your self-care plans. This might include helping to

monitor your signs and symptoms, helping to organise your medications, doing physical activity or exercises with you, or helping to remember things. Joining groups can also be very helpful as well as providing a bit of social activity.

Your facilitator should be able to tell you if there are any heart failure support groups or other heart disease support groups in your area.

Managing Heart Failure Quiz

*Please tick what you think is the right answer
(only tick 1 box per question)*

1. How often should people with heart failure weigh themselves?

- ☐ Once per week
- ☐ Now and then
- ☐ Every day

2. Why is it important that people with heart failure should weigh themselves regularly?

- ☐ because being overweight is unhealthy
- ☐ to check whether the body is retaining fluid
- ☐ to assess the right dose of medicines

3. Which of these statements is true?

- ☐ when I cough a lot, it is better not to take my heart failure medication
- ☐ when I am feeling better, I can stop taking my medication for heart failure.
- ☐ it is important that I take my heart failure medication regularly

4. What is the best thing to do in case of increased shortness of breath or swollen legs?

- ☐ call the doctor or the nurse
- ☐ wait until the next check-up
- ☐ take less medication

5. What can cause a rapid worsening of heart failure symptoms?

- ☐ a high-fat diet
- ☐ a cold or the flu
- ☐ lack of exercise

6. What does heart failure mean?

- ☐ that the heart is unable to pump enough blood around the body
- ☐ that someone is not getting enough exercise and is in poor condition
- ☐ that there is a blood clot in the blood vessels of the heart

7. Why can the legs swell up when you have heart failure?

- ☐ because the valves in the blood vessels in the legs do not function properly
- ☐ because the muscles in the legs are not getting enough oxygen
- ☐ because of accumulation of fluid in the legs

8. What is the function of the heart?

- ☐ to absorb nutrients from the blood
- ☐ to pump blood around the body
- ☐ to provide the blood with oxygen

9. Which statement about exercise for people with heart failure is true?

- ☐ it is important to exercise as little as possible in order to relieve the heart
- ☐ it is important to exercise and to rest regularly in between
- ☐ it is important to exercise as much as possible

10. Why are water pills prescribed to someone with heart failure?

- ☐ to lower the blood pressure
- ☐ to prevent fluid retention in the body
- ☐ because then they can drink more

11. Which statement about weight increase and heart failure is true?

- ☐ an increase of over 2.3kg (5lbs) in 2 or 3 days should be reported to the doctor at the next check-up
- ☐ in case of an increase of over 2.3kg (5lbs) in 2 or 3 days, you should contact your doctor or nurse
- ☐ in case of an increase of over 2.3kg (5lbs) in 2 or 3 days, you should eat less

Heart Failure Quiz: The correct answers

If you are unsure about any of these answers please talk to your facilitator.

- 1. How often should people with heart failure weigh themselves?**
☒ Every day
- 2. Why is it important that people with heart failure should weigh themselves regularly?**
☒ to check whether the body is retaining fluid
- 3. Which of these statements is true?**
☒ it is important that I take my heart failure medication regularly
- 4. What is the best thing to do in case of increased shortness of breath or swollen legs?**
☒ call the doctor or the nurse
- 5. What can cause a rapid worsening of heart failure symptoms?**
☒ a cold or the flu
- 6. What does heart failure mean?**
☒ that the heart is unable to pump enough blood around the body
- 7. Why can the legs swell up when you have heart failure?**
☒ because of accumulation of fluid in the legs
- 8. What is the function of the heart?**
☒ to pump blood around the body
- 9. Which statement about exercise for people with heart failure is true?**
☒ it is important to exercise at home and to rest regularly in between
- 10. Why are water pills prescribed to someone with heart failure?**
☒ to prevent fluid retention in the body
- 11. Which statement about weight increase and heart failure is true?**
☒ in case of an increase of over 2 kilograms in 2 or 3 days, you should contact your doctor or nurse

Adapted from: DUTCH HEART FAILURE KNOWLEDGE SCALE. M.H.L. van der Wal, T. Jaarsma. June 2003

Further Information

This section provides more information and ideas on topics which may or may not be relevant for you. Nevertheless, many of these topics can have a substantial effect on the quality of life of many people with heart failure.

Keeping Your Spirits Up

Having any long-term illness can have a big impact on your life – you may not be able to do all the things you could do before – you may feel unwell or tired a lot of the time – you may miss your old life and the way things used to be. This can lead to people with heart failure feeling low or depressed, particularly in the early stages – it can feel like your whole life has been turned upside down.

But it's not all doom and gloom. These feelings often change as time passes. It's all about adapting to your circumstances. To adapt successfully you need to find ways to make the most out of your life *as it is now*. This section provides some tips and ideas about how to check your mood and what to do if you are feeling down or depressed about your situation.

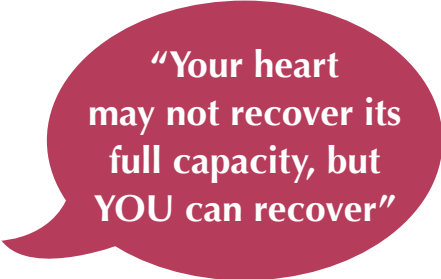
Is there a problem?

Feeling low down or blue from time to time is a normal part of life and usually this is nothing to worry about. However, if you are feeling low or depressed, or angry and frustrated a lot of the time, then it might be worth seeing if there is something you can do to improve your situation.

It is worth saying at this point that most people with heart failure are not depressed and many find ways to enjoy their lives despite any limitations that having heart failure puts on them. So there is hope!

The first step is to figure out how well you are doing. The common symptoms of depression are:

- feeling low or depressed, sad or empty
- feeling irritable or angry
- loss of interest and enjoyment in things you would normally enjoy
- change in appetite
- difficulty sleeping or sleep being more disturbed than usual
- mood swings or bouts of crying



“Your heart may not recover its full capacity, but YOU can recover”

- decreased activity – withdrawing from usual activities
- feelings of worthlessness or guilt
- reduced concentration – finding it hard to think straight or talk
- low self esteem and confidence
- pessimistic thoughts – for instance feeling that everything is pointless

If you feel low or depressed from time to time, then it may be worth reading the next few pages and trying out some of the ideas.

If most of the things above apply to you and this has been the case for two weeks or more, then you should mention this to your heart failure nurse or your GP, as there are treatments that could help.

If you feel that these symptoms are very strong to the point that they make your life unbearable or if you have got to the point where you often think about ending your life, then you probably have a serious bout of depression. If this is the case, please contact your GP or heart failure nurse straight away. There are both medicines and therapies that can make a huge difference for people with depression – indeed most people make a full recovery. OR, if you are not comfortable contacting the NHS, you can phone the Samaritans.

What can I do about it?

First of all, it is useful to understand the problem a bit better. This may help you to see ways around it.

How it works – low mood leads to low activity

As we said earlier, feeling low or depressed occasionally is normal and particularly if you have been seriously ill recently. For example if you have been in hospital recently with heart failure, the whole experience can be exhausting and can knock you sideways both physically and mentally.

A natural reaction to feeling ill is to withdraw from activities that you would normally do, such as meeting friends and family, doing things you enjoy and getting out and about in general. This gives you the space and time you need to recover and is fine. Up to a point. The problem is that withdrawing from things that you do also tends to affect your mood. It makes you feel more sluggish or tired, you are more likely to feel isolated and your mood generally sinks lower. The more you feel low or depressed, the more you withdraw from doing things that you might enjoy or that you find rewarding. Then of course your mood becomes even lower – it's a downward spiral. The picture below shows how this works.

"It is important to maintain a positive mental attitude; You may need to make compromises, but you can still have a good life"



Some people who are depressed say they feel like they are “looking up from the bottom of a black hole” and they can’t see how it is possible to get out.

A very important thing about the above diagram is the way that your feelings and actions can reinforce each other. It can become an unhealthy cycle, or a ‘downward spiral’ that is hard to break out of. These negative effects can spill over into other areas of your life. For example, giving up on physical activities not only affects your mood and how well you sleep, but your physical health as well.

The good news is that the ‘mood spiral’ works both ways – the way out of the black hole is to put the spiral into reverse! If you start doing more activities your mood starts to lift. Then you have more energy and are able to do more, which lifts your mood further. You don’t have to do this all at once of course – most people find that a step-by-step approach works best.

It can take a lot of willpower because you may not feel like doing things. However, once you start, you will often find that you enjoy them more than you thought you would. It will be much easier to stick at it if you make a plan.

Activity planning

Every day should contain some enjoyable activity

- Make a list of the days of the week and make sure there is some activity for each day – try to think of something that you will enjoy.
- For example, this might include going out, seeing friends or family, reading a book, listening to music, buying yourself a small gift or getting a bit of exercise.
- You may find that you don’t enjoy these activities as much as you would normally – Sometimes it takes a few goes before you start enjoying things again, so it is worth sticking at it. Also, you will probably feel

**“if you are
angry – go and punch
pillows/scream...
let it out”**

better than you would if you just sat around feeling miserable.

- If you can make these things into a routine, even better.

MON	Go to library
TUE	Buy new shirt
WED	Visit Bob Smith
THUR	Walk round park
FRI	Meet friends for lunch
SAT	Go shopping
SUN	Walk round harbour

You can also use the ‘Weekly Progress’ section in your Progress Tracker to keep track of your mood and see how it changes on days when you do more things, this might help you to see how doing a bit more each week can help to lift your mood.

An added bonus: physical activity

If you decide to do some **physical** activities, such as walking or gardening, these have extra benefits. When you do physical activity, it increases your energy levels and improves the quality of your sleep. Physical activity reduces stress and muscle tension and it can provide opportunities to interact more with other people, which many people enjoy. Physical activity also releases ‘feel-good’ chemicals in the brain. These can help to improve your mood, especially if you also enjoy the activity

itself. It can also provide a distraction from negative thoughts. Physical activity can also have an important effect on your heart failure symptoms and keep your muscles fit so that you do more in life.

How it works – Persistent negative thoughts

One of the most unpleasant things about depression is what it does to the way you think. People with depression often only see the bad side of things. They often report having very negative thoughts about themselves or their relationship with other people – thoughts like “I’m no good any more”, “I am always letting people down” or “My life is as good as over”. Negative thoughts can be images and memories as well as words. You may also tend to jump to negative conclusions about what other people do or say.

If you tend to look at things negatively (especially yourself, your future and the world around you), over time this can affect your mood. The more you think like this, the more likely you are to withdraw from a lot of activities that you would normally do. This might include meeting friends and family, doing activities that you enjoy and getting out and about in general. Then you will have even more negative thoughts and your mood becomes even lower – and we are back on the downward spiral (as in the diagram above). Over time these thoughts can become very persistent and often come to mind unexpectedly – they can become ‘automatic’.

So, how can you break the downward spiral and find your way back to a normal way of thinking and feeling? There are four simple steps which might be worth trying here:

1. Spotting the signs

The first step is to accept and acknowledge the thoughts you are having. Try to recognise when you have negative thoughts. What are the thoughts? It can be useful to make a note of these so you can think them through properly.

What situations trigger your negative thoughts? It may be that you can avoid these situations or prepare yourself for them in the future.

Negative thoughts often involve one of the following:-

- **Imagining the worst** (catastrophic thinking). For example, "I'll never be able to enjoy life again", "my life is over".
- **Exaggerating the negative** aspect of any slight setback. For example, if someone you know passes you in the street and doesn't seem to notice you, you might think "he doesn't want to know me anymore".
- **Feeling guilty**. YOU may blame yourself for all the bad things in your life – for example, "all this is my fault"; "I am just a burden on my family".

MIKA'S STORY:

"I think ACCEPTANCE is the first medicine of depression. It drove me batty when everyone kept telling me to be positive all the time ... when you're exhausted and feel horrible this isn't possible. So I'd say again ACCEPTANCE of the thoughts you are having is important. At first the thoughts might be worrying of course. But the main thing I learnt with being ill is the more you fight it the worse you feel – the more I tried to "be positive" and tell myself I had energy when I didn't, the worse I felt. It's an arm wrestle with yourself that you just keep losing. I like to describe it like trying to run against the waves in a very wavy sea. You will just get knocked down (the waves being the negative thoughts). If you dive with the waves (the thoughts) and accept the fact that you are having them, then it is easier and you can be more relaxed. It's all about GOING WITH THE FLOW. This doesn't mean accepting that the thoughts are true – it means acknowledge that these are the thoughts you are having, and then you can start thinking about your alternatives."

- **Ignoring positive things that happen.** For example, if a relative phones you up to ask how you are doing, instead of thinking “that was nice of her” you might think “she’s only calling because she thinks I’m going to die”.

2. Explore the negative thoughts

Ask yourself – is this idea really true?

- What would be a more realistic thought to have in this situation? Come up with some positive “self talk” – a statement that you can say to yourself when you find this negative thought arising. For instance, if you find yourself thinking “I am useless – I am letting people down again” if you are too tired to go to a family event, instead you might think “I need to take care of myself so I will have more energy tomorrow. They are good people and will understand”.
- If you tend to have negative thoughts in the same situation, is there a way that you can avoid that situation or prepare yourself in advance by using a bit of positive self-talk? For instance, if you tend to think more negatively when you are tired, you may be able to do some things to avoid getting over-tired.
- If you tend to think negatively when you are stressed, you may be able to use some of the relaxation techniques from the ‘Managing Stress’ section earlier in the manual or from the relaxation audio to

“Having heart failure has made me realise what’s important in life. I don’t fret about the little things now”

reduce your stress levels. The relaxation and mindfulness exercises in the Stress Management section can also help people to first accept and then distance themselves from persistent negative thoughts.

- Remind yourself that although sadness is unpleasant, it is not dangerous. Often these thoughts will fade away over time.
- If your negative thought is about what other people think about you, turn the problem around – if somebody else was in your situation, what would you think of them. For instance, if you think you are being a burden to your family, consider what you would think about them if they had heart failure – would you see them as a burden, or would you want to help them?
- Find ways to test your negative theory – instead of jumping to your own conclusions, you could ask other people how they feel for instance.

There are some more examples of negative thoughts and ways to challenge them in the table below.

3. Figure out what works for you

It doesn’t always work first time. Like a lot of things, learning how to deal with negative thoughts takes a bit of practice. If something that you try doesn’t work, don’t see it as a

“It’s about listening to yourself and doing what’s right for you”

failure, just try a different approach next time. The key thing is to learn from your experiences. Try out different ideas about what you can say to yourself or how you can avoid the situations which lead to negative thinking. Over time try to learn what works best for you.

If there is a particular situation that often triggers negative thoughts or makes you

upset, it may be worth making a plan in advance about what you will do next time this happens. For instance, you might say, if I get tired in the next week and I notice that I am starting to have negative thoughts, I will take a break and do my relaxation exercises (or have a nap) and see if that helps.

Negative thinking	Alternative ideas
This heart condition is really serious – this is the end of the line for me	Am I exaggerating this? • Heart failure is a challenge, but it is still possible to find happiness. • Many people with the same condition find ways to cope. Try some positive self-talk: • I need to focus on what I can still do and find ways to enjoy my life as it is now • Feeling low can be caused by the symptoms of heart failure. If I focus on managing the condition, things might get better • I will adjust to my new reality • I’m not dead yet!
My family used to rely on me, but now I’m no use to anyone	Is this really true? • OK, things have changed and, but there are lots of things I can still do. Let’s turn this around ... How would you react if one of your friends or family said that they were in this situation? • Maybe you would re-assure them that you still care about them. • Maybe you would want to help them for a change? • Maybe your family feel this way about you?

I can't be bothered to go out for my walk – what's the point?	Maybe you are thinking this way because you are feeling low or depressed. Remember that withdrawing from activities is the sort of thing that can make your mood even lower. • Going for a walk often helps me to relax and clear my mind • Even though I don't feel like it, I'll give it a try and see how I feel after the first 5 minutes
You ask your partner if he wants to go out tonight and he says he is tired. You think "he doesn't want to go out with me because I am no fun. He doesn't love me anymore".	Is there another explanation? Am I jumping to conclusions? • Maybe he is tired like he said. Is there a way to test this? • Let's see if we can go out another night – I'll ask him.
My friends won't want to do things with me anymore. I will just slow them down or be in the way.	• I may not be able to go out as much as in the past, but I will tell my friends and they will understand. • I will organise to do more things at home – sometimes with my friends, sometimes without them. Let's turn this around ... How would you react if one of your friends said that they were in this situation? • You might tell them not to be so crazy – of course you still want to do things with them. • You might be happy to change some of the things you do. Being friends is more important than what you do.
I can't cope with living like this. It would be better if I was dead.	If your thoughts are this dark, then you should put down this manual and seek help from your GP – there are both medications and therapies that can help you turn this around.

4. Look after yourself generally

The physical effects of heart failure like tiredness and sleep problems can all contribute to feeling low. So, the better you can manage your condition and keep the symptoms under control, the better you will feel. You can use the ideas in the rest of this manual to help you keep your heart failure symptoms to a minimum.

If you have any concerns or difficulties in managing low mood, please discuss them with your facilitator or heart failure nurse or your GP – they are there to help! He or she may be able to prescribe medicine that can help, or arrange for you to see a specialist who can help you to control the negative thoughts. Often you can be seen in a matter of weeks.

Other tips and tricks for managing low mood

EXPECT TO HAVE GOOD DAYS AND BAD DAYS

Hopefully after a few weeks you will notice a gradual improvement in your mood. However, you will also have bad days, and this may

affect how you use your programme or engage with the activities. This is all perfectly normal and to be expected.

DON'T OVERDO IT

Slow and steady is often the best strategy. Think about breaking things down into small, manageable chunks – that generally makes things easier.

INVOLVE FAMILY AND FRIENDS IF YOU CAN

Like all of us, when you are feeling a little down, you may have found that just having others around can be helpful. They may help you look at things differently, find ways to solve problems or maybe just be there for a chat. Getting others involved isn't for everyone and you may not be ready to take this step yet – don't worry if that is the case. But if you think you might find the support of others helpful and they want to help then why not ask? If you show them these pages, and the Family and Friends Resource they may find them helpful too.

Getting Support From Others

People are often reluctant to ask for help. However, help is available and you are not alone. Help may come from family, friends, work colleagues, voluntary organisations, and social services. Help or support may include:

- emotional support or encouragement
- practical support such as giving lifts, cooking, giving time
- support in getting information or making plans

People who get help from others often cope better. You may be worried that people will think you can't cope or that you have 'failed' in some way, or that you don't want to be under an obligation to anyone or become a burden to someone else. However, giving support can provide peace of mind and be rewarding for the person providing the support. It sometimes helps to think about this the other way round – if you had a relative or a good friend who was ill and struggling cope, would you want them to tell you about it or would you want to do what you can to help them?

"I have always been a bit independent and stubborn ... but you need to ask for help, and that is really hard ... Even if it's from a really close relative or a friend, it's really hard to make yourself ask for help. Once you've done it, it's like: Well I'm glad I did that"

Tips for getting help

- Take a little time to think about and identify all the people who you think may be able to help you.
- The first time you ask is the hardest but it will get easier. Sometimes people may be waiting to be asked as they may not know what help to offer. Practical tasks include cleaning, laundry, putting out dustbins/recycling, shopping, providing transport or just being around to talk to.
- Start with asking for help with a small task such as getting a few items of shopping while they are out anyway. Once people know what you need, over time, help may be offered without you needing to ask.
- If you turn down help it may not be offered again. Perhaps you could think of a simple task to be completed.
- Organisations such as local carer and support groups can be valuable support.
- Keep in touch with friends and family via telephone, e-mail, text messaging, Skype, Face time or other social media such as Facebook. You may have someone in your family or a friend who can help you set this up. Friends you know through clubs and organisations may also help.

*"I can just sort of say
hello without being on the
phone for three hours"*

Organising your support

Once you have a better idea of who can help you and what they may be able to do, it may help to plan how you are going to involve your friends and family. You might like to use the table below.



Example

Who can help?	How can they help?	When can they help?
Relative	Shopping	Once a week - Thursday evening
Neighbour	Pop in for a chat	Once a week - Saturday morning
Alice (friend)	Meet with me for coffee/tea etc.	Once a week - Friday afternoon

Getting help from social and council services

This section is about what support you and any person who may be helping with your care may be entitled to and how you can find out more. This includes information about financial benefits, care at home services and about equipment/aids (such as grab rails, bath or shower aids, perch stools and mobility aids).

It is worth noting that the organisation responsible for providing equipment and other

services may vary in different places, but **your facilitator should be able to advise you about who to contact locally to have your support needs assessed.**

In most places in the UK, you can apply to your local Social Services for a "needs assessment" (England and Wales) or 'community care assessment' (in Scotland) or a 'Health and social care assessment' in Northern Ireland. This can include home care help with things like cleaning and shopping and access to day centres to give you or the person who cares for you a break, access to

other activities or groups in your community. Where appropriate, your home can be assessed for appropriate aids and equipment. This could include, for example, grab rails, bath/shower aids, perch stools and mobility aids.

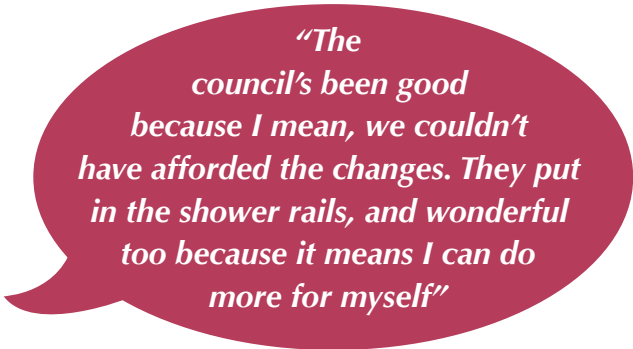
You can also ask for an 'advocate' to assist with completing forms for any benefits that you may be entitled to. Ask your facilitator for websites or contact numbers to find out more.

Carer Assessment

If someone helps with your care, like a partner or family member, they have a legal right to have a 'carer's assessment'. This is also available through the Social Services

department of your local authority or council. This involves assessing what help they might need. The Social Services assessment is used to decide what help to provide. The assessment usually involves a visit at home from a social worker or sometimes another health professional or trained volunteer.

During the carer's assessment you can also ask for care at home services or a sitting service that will allow your caregiver to have some time for him/herself. The assessor will know what is available in your area. Sometimes these are provided by voluntary agencies or charities.



"The council's been good because I mean, we couldn't have afforded the changes. They put in the shower rails, and wonderful too because it means I can do more for myself"

Common Sources of Stress in Heart Failure

It is all too much to think about. I wasn't expecting this

Even if you have had symptoms for some time, the diagnosis of heart failure may have come as a shock. For many people the term heart failure is frightening as they think this means there is nothing you can do about it. This can be misleading as there are many things that can be done to manage the condition. You may be blaming yourself and be feeling angry. You may have concerns about your work, family, or finances and what this all means for the future. All these concerns are common and frequent sources of stress. This is where finding out about heart failure and what you can do about it can help with stress. Some people try to cope with all the uncertainty by putting the problem to the back of their minds. However, this can be unhelpful, as you might overlook ideas that can be helpful in managing your symptoms or that might give you more energy. The clearer the picture you have of your own condition and circumstances the more you will feel in control and able to cope.

I feel I need to try and make sense of it all

Most people feel the same. They want to know what heart failure is and what it means to them personally. You might also question how this will affect your relationships and social life and of course your future health and life expectancy also come to mind. Having heart failure can cause great disruption to the way you live and the way you think about yourself – it can present a number of challenges. For most people, making sense of all this takes

time but as you understand more about what heart failure is and how you can manage it, the easier it will be for you to deal with. This manual and your facilitator will help you with these things, as well as helping to manage your stress levels.

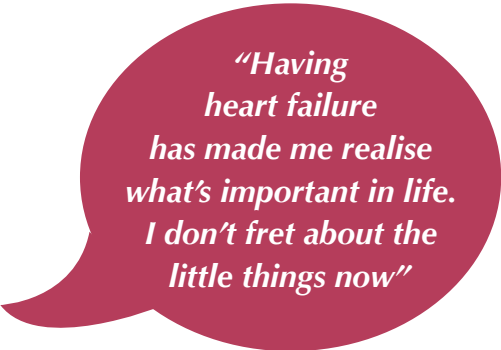
I don't feel as if I am the same person anymore

Having a long-term condition can mean that you are no longer able to fulfil the roles you previously performed as part of your daily life. For example, not being able to do your job, look after children or grandchildren or go out with friends and family. This can make you feel as though you have lost your sense of purpose – as if it has changed who you are. This can be challenging and stressful and it is a common experience. If you have been used to always looking out for others before your own needs you may find it difficult to let go at times when you need to, or to let other people help you. These changes can be difficult to accept. However, many people find that making small changes and re-thinking what is important to them in life allows them to achieve a greater sense of control and feel more at ease with themselves.

People are trying to take over

The first reaction of people who you know may be to try and protect you. This is often because they are concerned and also perhaps because they are not sure about what heart failure is and what it means in terms of daily life. If they are being over protective and trying to 'wrap you up in cotton wool' they may not understand that keeping up the

activities that you are happy and comfortable with is actually good for you, both physically and mentally. However, it is also important that you accept help when you need it. Communication is the key to getting the balance right – talk to the people around you to let them know what you need, how they can help and what you want to do for yourself. By sharing work such as household chores you will find you can do more of the things you need and want to do. Don't try to be 'Superman' or 'Wonder Woman'. Try and resist being too proud and accept help when it is offered such as carrying heavy washing so you can save your energy for when it is needed.



"Having heart failure has made me realise what's important in life. I don't fret about the little things now"

Encourage your partner, or anyone who you think might be able to help, to read the REACH Family and Friends Resource as this will explain more about the condition and its effects on daily life. For instance, it will help them understand your need for doing physical activity to keep your heart healthy.

I find it stressful coping with the changing symptoms

Many people find it difficult to cope especially

when symptoms may be unpredictable with good days and bad days. This manual has a number of tips and ideas that might be helpful here. For instance, if you keep track of how your symptoms change over time, this will help and give you a better understanding of what makes your symptoms better or worse. Pacing your activities and having a plan for coping with days when your symptoms are worse will also help. Feeling anxious or stressed about your symptoms may make things feel worse. If this happens, then using some stress management exercises, such as the breathing and relaxation exercises in the 'Managing Stress' section may be helpful.

I'm scared to do anything in case my symptoms get worse

People are often afraid of doing anything that might make their condition worse, or cause damage to their heart. This can lead them to avoid the very activities that may help symptoms and improve things for their hearts. This particularly applies to physical activity. The fact is that following a paced (slowly building up) walking or exercise programme in combination with your daily living activities will help to build your strength and stamina and is actually good for your heart.

Having a fear of activities is very common with people with heart problems. As well as losing fitness, avoiding activities that you used to enjoy may lead to feeling stressed and depressed, or feeling isolated. Your facilitator will be able to help, so it is important to discuss these feelings with him or her. Please see the 'Keeping Active' section earlier in the

manual and ask your facilitator to show you how to safely pace your activities and to avoid doing too much.

I don't know what to do now I have stopped work

Stopping work when it is not your choice can be stressful. You may feel that your work gave you purpose in life or a reason to get up in the morning. This change may also leave you with financial worries. At the time, these changes may cause a great deal of stress. Discussing your concerns with your facilitator can help you deal with this. They may be able to offer you some practical advice and direct you to other types of support such as local community groups and occupational health services, as well as suggesting some of the financial benefits you may be able to apply for.

I don't want to be a burden on others

People often have concerns that their family, or other people who provide care will have to take on too much. It may be that you do not want those close to you to take on personal care such as helping with going to the toilet or washing. You may be able to get other professional help with this so that you don't need to ask as much from your caregiver. If possible discuss this with your caregiver, then let your GP or facilitator know that you would like to seek additional outside help.

I find the symptoms frightening and stressful

Symptoms such as shortness of breath or dizziness can be stressful and sometimes frightening. Often becoming anxious can

make you even shorter of breath. Learning to relax and practising breathing exercises when you are feeling OK can help you to control your breathing when you experience these symptoms. Please read and practise the exercises for relaxation in the Managing Stress section and see the Managing Breathlessness section.

Other methods can be useful in coping with these episodes such as positive thinking and visualisation. You may want to try the positive thinking exercises and the visualisation exercises on the relaxation audio.

I get annoyed when I get so tired from simple daily living tasks

It can be very frustrating when you feel that you cannot complete all that you want to in a day. Pacing and energy-saving steps will help you here. For instance, sitting using a high stool to do tasks like washing up or preparing food may help. Having heart failure can mean that you can get really tired if you over-do things. One way to cope with this is to manage how much energy you use so that you don't get too tired (for example by going at a steady pace, or breaking big jobs down into smaller tasks). We have some tips and ideas about how to do this in the 'Managing Changes in Your Symptoms' section. Many of these have come from other people with heart failure.

All the concerns above are very common and your facilitator can also help to discuss which ideas might work best for you.

Managing Your Finances

When you developed heart failure, you, or someone who helps to care for you, may have had to reduce your paid hours or give up your job. You may also be faced with additional costs, such as transport, heating and laundry. There may be benefits and/or tax credits available to you, such as a reduction in council tax.

Whether you can claim benefits depends on several factors including your income, savings, your age, what National Insurance contributions you have made, whether you are disabled and if you have children. Each particular benefit has its own rules and the benefits available may vary depending which country you live in.

Filling out forms can be a difficult and complicated process. However, you can contact your local council and ask for someone called an 'advocate' to assist with completing forms for benefits. This can also be set up as part of the carer's assessment (see the section on 'Getting Support' above). Alternatively organisations such as the Citizens Advice Bureau, Carers UK or Age UK can also help. If you live in social housing, you can contact your local council's housing officer for support.

If you are entitled to one benefit you may find that it can lead to others. This may include free dental care, an eye test or prescriptions for example. You may also be able to claim for transport to and from hospital appointments.

Apply for benefits as soon as possible because many benefits cannot be backdated. If your claim is turned down or if you receive less benefit than you feel you are entitled to, you can appeal.

Other sources of financial help

Benefits are not the only way of getting financial help. You may be able to apply for:

- Help with health costs, for example to pay for dental treatment, spectacles or prescriptions.
- A grant to help adapt your home.
- Charity grants or benevolent funds such as the Royal British Legion if you have served any time with the armed forces.

Managing Your Home and Work Environment

This section is about how you can balance the hassles of home or work with managing your heart failure.

Home

Top Tips

- To make planning and organising a bit easier, you could use a calendar or diary to show when activities should be completed.
- Consider prioritising tasks. What do you feel are the most essential or important tasks that need doing? What things are less important and can wait? It may help if you discuss this with any person you live with, or who comes in to help you.
- It is sometimes easier to break tasks down into easier steps instead of trying to do everything in one go. Be patient as some tasks might take longer.
- If you have a computer, you could consider online shopping or alternatively some shops offer a delivery service.
- Involve other people in household. For example, ask for help from a friend, neighbour or relative to help you with preparing meals, or shopping.
- Consider making use of new technologies such as voice activated or automated control of lighting, heating, door answering systems, and calls for help.
- Try to be as flexible as possible and 'go with the flow', making the most of 'good' days.

"That calendar up there is our bible... If he wants to know anything he looks on there. I write everything on there. If I'm at work, if I'm not at work, when I'll be home, when we're going to the hospital, the doctor's appointment, everything goes on there"

You may be able to get help with household tasks such as putting out the dustbins and recycling from your local council.

Work

If you are working, you should inform employers as soon as possible about your situation as they may be able to help you in several ways – for instance, they may be able to change your work pattern or set things up for you to work at home. Often people with heart failure who go back to work can arrange a gradual re-start. Start by talking it over with your manager or occupational health. If you belong to a union they will be able to offer you advice too.

Many large organisations also have a policy on the rights of caregivers. If you have someone who helps to care for you, they may be entitled to special leave for caregiving activities.

Travel

It is really important to make sure that you have enough medications (and some spare medication) to last for the duration of your trip. So plan your trip well.

If you are flying, keep your medication in your hand luggage in case you are delayed or your luggage is lost. You might also wish to arrange airport assistance to help you get to where you need to be on time.

When travelling, carry a written medical history and a list of your current medications – you may need it if you have to seek any medical help during your trip.

If you are going abroad, you should check in advance what rules apply to taking your medications into any countries that you are passing through, as some medicines used in the UK may be controlled in other countries.

You should also check with your GP if you are OK to fly – flying does put some stresses on the heart and the rest of your body and may not be advisable if your symptoms are not well controlled.

Similarly, travel to high altitudes (1500 m or more) and travel to very hot and humid destinations are not advised unless you are in a good, stable condition.

You may also need some vaccinations depending on where you are planning to go. Your GP surgery will have a nurse or other health care professional who is an expert on travel vaccinations. You should start to organise this at least 8 weeks before you are due to travel, because some vaccinations need to be given well in advance to allow your body to develop immunity and some involve multiple doses spread over several weeks.

Remember to keep monitoring your weight and any swelling to make sure you can spot any fluid build-up early and take action (using your Traffic Light Action Plan). Make sure that you drink plenty of water, especially if the weather is hot or humid. Finally, pace yourself – don't take on too much in each day, but do have a great time!

Check with your GP surgery to check that you are fit to fly and if you need to make any special arrangements to travel abroad. Allow plenty of time (at least 8 weeks) before travel for the GP to get your vaccinations organised.

Sex

For most people with heart failure it is quite safe to enjoy sexual relations as long as your symptoms such as shortness of breath and chest pain are controlled and not bothersome. After a diagnosis of heart failure, you may feel anxious and depressed, making you less interested in sex. It is not uncommon in men with heart disease to suffer from erectile dysfunction (impotence). Some drugs such as beta blockers can also have side effects which can worsen these symptoms.

Don't be embarrassed to discuss any concerns you have about sex with your doctor or nurse - they are used to discussing this kind of thing.

The following suggestions have been adapted (with permission) from the Heart Failure Matters website which is endorsed by the European Society of Cardiology:

- Choose a time for sex when you are rested and relaxed.

- Avoid having sex immediately after eating a heavy meal or binge drinking.
- Have sex in a comfortable and familiar room.
- You can use foreplay to help your body get used to the increased level of activity.
- Use positions that you find less strenuous such as lying on your back or side.

It is also safe for people with heart failure who have had a device such as an ICD (implantable cardioverter defibrillator) fitted to have sex. You can get more information about ICDs and sex and heart disease from the British Heart Foundation website.

If you have any concerns mention it to your doctor or nurse. It may be possible to help you enjoy a rewarding sex life.

Sleep

Many people living with heart failure experience poor sleep. However, there are things that you can do to improve the quality of your sleep.

Getting a good night's sleep is important for your mental well-being, for your energy levels and to keep the symptoms of heart failure under control. Most people sleep for around 7-9 hours per night. Sleep patterns may change as you age; older people frequently sleep less at night but may have daytime naps.

Poor sleep is often a sign that something is not quite right. Poor sleep can make you feel irritable, depressed or sleepy during the day. You may also experience headaches or an

upset stomach. You may also find it difficult to concentrate.

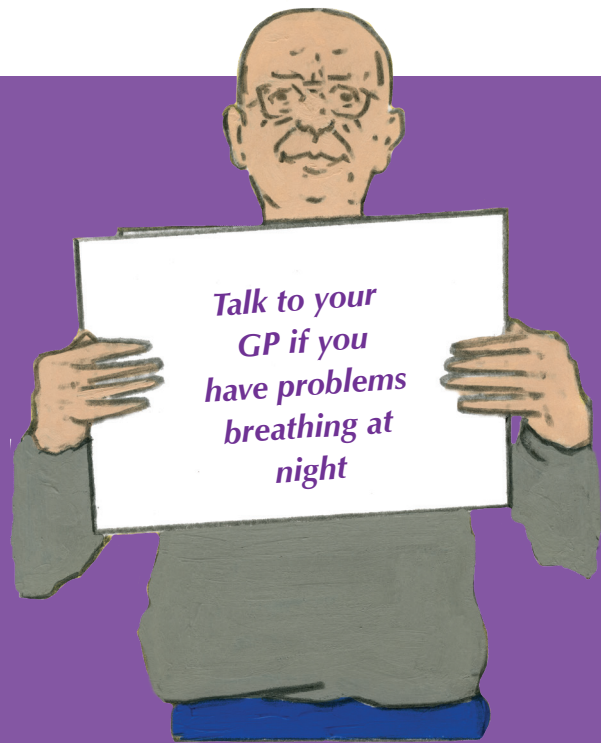
Poor sleep can have a number of causes including:

- worry, anxiety or stress
- breathlessness due to heart failure
- pain from other health conditions such as arthritis
- side effects of medication, including disturbed dreams or cough.
- smoking
- alcohol use

Suggestions for sleeping well

- Establish a good bedtime routine. Try to do the same thing every night before going to sleep, as this will help train your body to sleep at night. For example, read a chapter of a book before going to bed.
- Get up at the same time of day.
- Keep the temperature of the room at a constant level, not too hot or too cold.
- Keep a sleep diary to see if there is a pattern or reason why you are not sleeping well and to help plan your new routine. You can use the Progress Tracker to rate your sleep quality.
- Try eating a light snack before going to bed, unless you suffer from indigestion. A glass of warm milk or a bowl of cereal for instance.
- Use relaxation or visualisation techniques or listen to relaxation music. The relaxation audio which comes with this manual is ideal for this.
- Listen to some music on the radio.
- If you have trouble getting to sleep, don't go to bed too early. This makes the night much longer.
- Avoid napping during the day. Or, if you need to nap, keep it short.

- Avoid alcohol and caffeine in the two hours before bedtime. These make you wake up earlier. Alcohol may also affect your safety if you go to the bathroom at night.
- Avoid smoking as nicotine also affects the quality of sleep.
- If you wake in the night feeling worried, try to avoid 'clock watching' and tossing and turning. Instead, sit up in bed reading or spend time out of bed doing a mundane chore or watching television. Write down everything that's on your mind and what you might be able to do about it. Go back to bed when you feel tired.
- Ensure that your bedroom is dark and quiet. Use a blackout blind if affected by light.
- If you have trouble getting to sleep, get up and go to another room and sit quietly for about 20 minutes before going back to bed. Consider separate beds or a separate room if you are disturbing your spouse or partner and the home situation allows.
- Exercise as directed in the 'Keeping Active' section earlier in the manual – this can really help to improve sleep. However, avoid exercise in the two hours before bedtime.
- If you are breathless at night you may sleep better propped up with some pillows. If this is a new symptom then you should report it to your GP or heart failure nurse specialist.



- If you have pain at night, either from your heart or due to another condition such as arthritis, discuss this with your GP. There may be other treatment options for you.
- Any side effects of medication that affect your sleep should also be reported to your GP who may be able to change your medications to prevent this.
- If you frequently stop breathing whilst asleep, discuss this with your facilitator or GP or heart failure specialist, as this can often be treated.
- If poor sleep continues then you should consult your GP who may be able to assess your sleep and recommend further tests or treatments. Sleeping tablets may not be an option because of your heart condition.

Implanted Devices

Not all people with heart failure need a cardiac device; they are for specific reasons such as correcting the rate at which the heart is beating. Devices include:

- Pacemakers – to monitor the heart and control your heart rate if it drops too low.
- Implantable cardioverter defibrillators (ICDs) – to monitor the heart rhythm and shock the heart if the rhythm is abnormal or the heart stops.
- Cardiac re-synchronisation therapy – a specific type of pacemaker that ensures the different parts of the heart contract in the right order. It may also have a defibrillator.

Many people with heart failure and their families or supporters are fearful that their device may ‘go off’ causing them to worry about going out alone. Some pacemakers make a ‘ticking sound’.

“I have gone out with her each time. She’s going to do some shopping today, which is a major step forward. That will be the first time that she’s done it.”

Tips for coping with devices

- If you are concerned about going out, start with short trips out with another person. Start with half an hour and build up the time. You will gain confidence over time. Reflect on how you feel after a trip out.

- If going out alone, take a mobile phone for peace of mind. You might like to use text messaging to keep in touch.
- Remember that if the device does go off, this is a good thing – it means that the device is working and doing the job it is meant to do.
- Follow the recommendations of your health care team about what to do when it fires.
- Add the contact details for the specialist or technician for your device to your list of contacts in your mobile phone or telephone address book. You can contact them for non-emergency advice.
- Consider wearing a medic alert bracelet or tag necklace. Ask your nurse or GP if you don’t know where to get these items.

For further online information about devices you can search the internet for:

- **NHS Choices (known as ‘NHS Inform’ in Scotland).** Heart failure treatment.
- **British Heart Foundation.** Pacemakers or defibrillators.

Your facilitator can also give you additional information. **Please do ask questions if you have any concerns.**

Alcohol

Alcohol can affect your heart, and drinking too much can worsen your heart failure.

For most people it is safe to consume up to 1 to 2 drinks per day. It is recommended (for both men and women) not to regularly drink more than 14 units a week, with at least 2 alcohol free days in the week advised.

People with more severe symptoms may be advised to keep off alcohol altogether.

If you have been diagnosed with alcohol induced cardiomyopathy, (i.e. your heart failure is as a result of excess alcohol intake), you are advised to abstain completely.

All people with heart failure are strongly

advised to avoid binge drinking and to stick within the recommended limits.

Drinking more than the recommended limits may:

- cause high blood pressure
- damage your heart muscle
- cause problems with your heart rhythm
- worsen ankle swelling
- make you put on weight

Discuss your alcohol intake with your facilitator or GP or nurse to find out what is safe for you.

1.5 units  Small glass red/white/rosé wine (125ml, ABV 12%)	2.1 units  Standard glass red/white/rosé wine (175ml, ABV 12%)	3 units  Large glass red/white/rosé wine (250ml, ABV 12%)	2 units  Pint of lower-strength lager/beer/cider (ABV 3.6%)
3 units  Pint of higher-strength lager/beer/cider (ABV 5.2%)	1.7 units  Bottle of lager/beer/cider (330ml, ABV 5%)	2 units  Can of lager/beer/cider (440ml, ABV 4.5%)	1 unit  Single small shot of spirits* (25ml, ABV 40%)

Many people get confused when trying to calculate units. Have a look at the guide below to see the alcohol content of your favourite drinks.

A single unit is 10ml of pure alcohol. ABV (alcohol by volume) is the percentage of the liquid that is pure alcohol.

So, a small glass of wine (125ml) at a strength of 12% has 15ml of pure alcohol. This is the

equivalent of 1.5 units. A large glass (250ml) contains 30ml of alcohol (3 units).

A Unit Calculator and Drinks Tracker are available to download from the NHS Choices (or NHS Inform in Scotland) website.

The information above has been adapted with permission from 'An everyday guide to living with heart failure' published by the British Heart Foundation.

Smoking

Why should I stop smoking?

You might think that now you have heart disease it is not worth quitting smoking, or that it is too late to get any benefits. However, nothing could be less true. When you smoke, your heart has to work harder to supply your body with sufficient oxygen. This is because smoking increases the carbon monoxide in your blood, which reduces the amount of oxygen carried by your red blood cells. It also makes your blood sticky, which reduces the benefits of aspirin or other blood-thinning medications you are taking. Smoking also leads to a narrowing of your arteries, which can increase your blood pressure and means your heart has to work harder. Indeed, many of your organs will have to work harder to remove all the toxins that smoking puts into your body.

So, the best thing you can do to help your heart condition is to stop smoking. You may have tried before. Don't be put off by previous unsuccessful attempts – most people have to try a number of times before they successfully quit smoking.

Tips for stopping smoking

Set a quit date and stick to it. Make sure family and friends know you are planning to stop and ask for their support. If household members smoke, encourage them to quit at the same time, or to consider smoking outside. Remove tobacco products from your home and places where you spend a lot of time.

Some people find using nicotine replacement,

like nicotine gum or patches or e-cigarettes, for a while helps them to quit smoking. There are also tablets (Bupropion and Varenicline) which can help decrease cravings. You can ask your doctor or the NHS Stop Smoking Service for advice about what might be most suitable for you. Using nicotine replacement or stop smoking medication really increases the chance that you will successfully quit smoking.

Keep a jar and put the money you would have spent on cigarettes or tobacco into it. Buy yourself a treat with the money you've saved.

Try to be more active, as exercise helps to reduce nicotine withdrawal symptoms.

Some people find that listening to relaxation audio, or using mindfulness techniques, can also be helpful in reducing their stress levels and reducing cravings.

You may find that chewing sugar-free gum helps, as it gives you something to do with your hands and mouth.

Cravings come and go – often they will go away after a few minutes – so find things that you can do to distract yourself if you have a craving. People who have successfully stopped smoking say that the cravings do go away completely over a few months, so don't think that you will have to live the rest of your life feeling desperate for a cigarette.

Remember, cutting down is harder than quitting smoking and you won't get the full benefits unless you completely stop smoking.

If you slip up, don't leave it a while, get straight back on track. It often takes several attempts to be successful. Figure out what led to you start again and think about how you could stop that from happening if you were to try again.

The NHS has a free smoking cessation service that can help you quit – this usually includes counselling as well as nicotine replacement or drug-based therapies. High quality research shows that using these services trebles your chances of quitting compared to doing it yourself. Ask your GP or nurse for contact details or look them up online.

A word on e-cigarettes: These are less harmful than smoking tobacco, as these only contain nicotine, without the hundreds of cancer-causing chemicals in cigarette smoke. Using these as a way to gradually cut down and quit is similar to using nicotine patches.

Finally, if you do decide to stop smoking, keep an eye on your diet and the amount you eat. Stopping smoking slows down your metabolism (as your body no longer has to clean out all the toxins from cigarette smoke every day). This means you can put on weight. Indeed, the average weight gain, one year after stopping smoking, is around 1 stone (6-7kg). So you may want to adjust your eating patterns to keep this under control.

What next?

Make plans to quit smoking, consider attending a stop smoking service.

Set a quit date and stick to it.

Once you've stopped, take it one day at a time and congratulate yourself on your achievement.

Healthy Eating

Eating healthily is important if you have heart failure because:

- what you eat can affect your blood pressure and your symptoms
- it can affect how much energy you have over the course of the day
- it can affect your risk of heart attacks and other health problems
- eating too much salt can make you retain more fluids

What is a healthy diet?

The recommendations for healthy eating for people with heart failure are the same as for the general population. This means eating a wide variety of foods and having the right amount of food and drink to maintain a healthy body weight.

A healthy diet is one that is low in fat (especially saturated fat) and includes plenty of fruit and vegetables, balanced with carbohydrates (starchy foods such as potatoes, rice and pasta) and protein (from things like meat, fish and lentils). Along with some milk and dairy foods, this will give you all the nutrients you need. You should also limit the amount of drinks and snacks you have that are high in sugar or salt or both. If you have heart failure you should be particularly careful not to eat too much salt (see the next section on 'Managing Your Salt'). So a healthy diet should include ...

- Fruit and vegetables – this should account for around a third of what you eat.

- Starchy foods, such as bread, rice, potatoes and pasta. Choose wholegrain varieties whenever you can, or eat potatoes with their skin on for more fibre. Again, this should account for around a quarter to a third of what you eat.
- Protein. This can come from meat (ideally lean meat or without skin), fish, eggs and beans, and from milk and dairy foods and soya products.
- Only small amounts of foods that are high in fat or sugar.

Most people in the UK eat and drink too many calories. They eat too much fat, sugar and salt, and not enough fruit, vegetables and fibre.

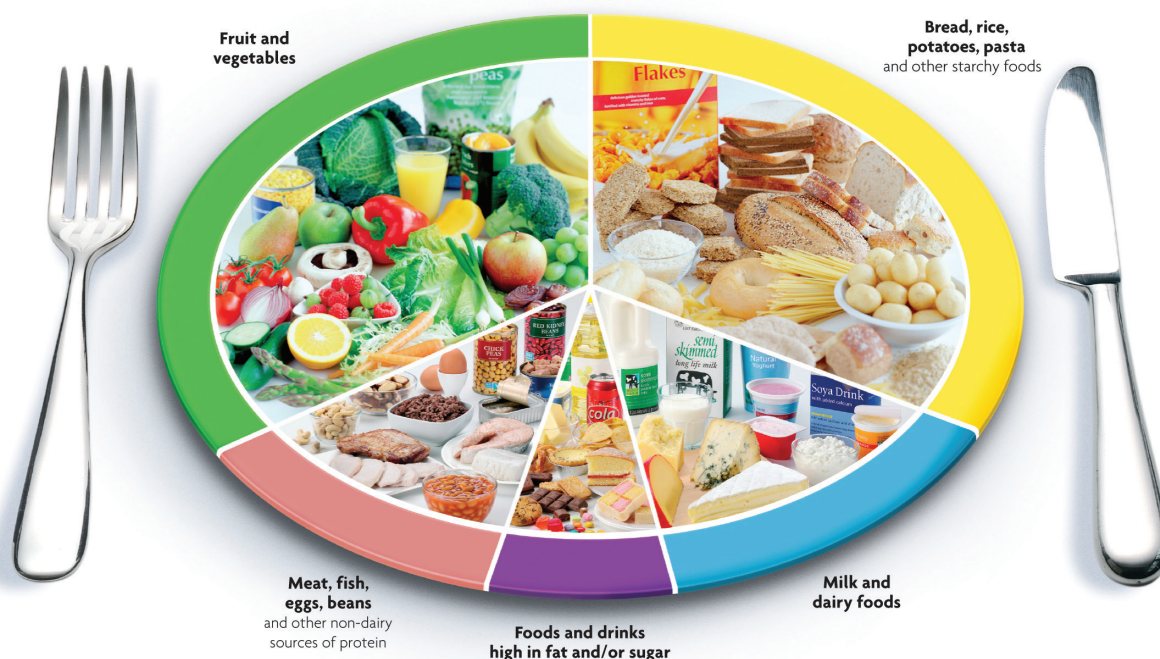
Some fats are more unhealthy than others – these are called **saturated fat** and **trans fat**. Saturated fats are found in meat (especially red meat), butter, cheese and other animal products. Trans fat is present in processed foods including many snack foods like crisps, biscuits, cookies, cakes, pies, pizzas, and deep fried foods, such as chips or chicken nuggets, as well as in some 'ready meals'. This type of fat is so dangerous and unhealthy that a number of companies are starting to ban it. It is worth checking out the labels of foods or snacks that you eat frequently to see if they contain trans fat.

You can get guides to food labels from a number of places on the internet including the British Heart Foundation.

The eatwell plate



Use the eatwell plate to help you get the balance right. It shows how much of what you eat should come from each food group.



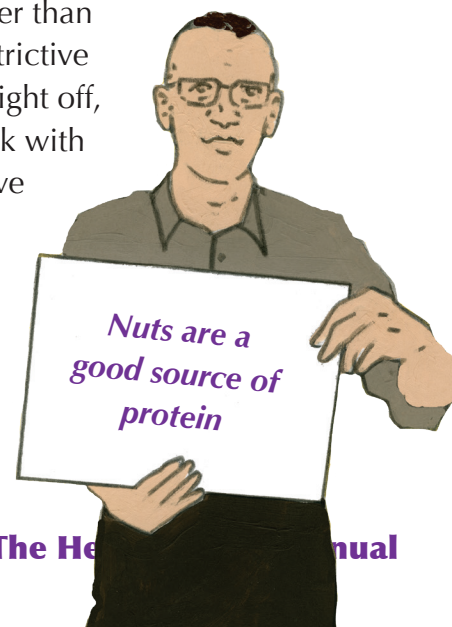
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If you have heart failure, then being a little overweight is not necessarily a problem. However, if your body mass index (your weight in kilograms divided by your height in metres, then divided by your height in metres again) is more than 35, this can cause some problems in terms of:

- putting extra strain on your heart and lungs
- putting pressure on your lungs when you lie down, making it harder to breathe and to get a good night's sleep
- making it harder to do exercise or to stay active
- increasing your blood pressure

- putting pressure on your knee and hip joints, which may make any joint pain a lot worse

If you are interested in losing weight, it is better to make a number of small changes that are sustainable, rather than trying to go on a restrictive diet. To keep the weight off, you will need to stick with any changes you have made – and if you want to lose more weight, you will need to add more changes.



Healthy eating tips

1. Limit the amount of fat you eat, especially 'saturated' fat.
2. Include plenty of 'starchy' foods in your diet – bread, rice, potatoes, pasta and choose wholegrain when you can.
3. Include food that is high in fibre such as pulses (beans and lentils) and whole-grain options. High fibre foods keep you feeling full for longer as they release their nutrients slowly.
4. Have at least five portions a day of fruit and vegetables.
5. Limit sugary drinks and unhealthy snacks (high fat or high sugar snacks).
6. Limit the amount of salt in your diet (see section on 'Managing Your Salt' later on).
7. If you drink alcohol, drink in moderation.
8. Fish is a good source of protein with low saturated fat. Aim to eat at least two portions a week, including at least one portion of oily fish such as salmon, mackerel, trout, herring, fresh tuna, or sardines.
9. Changing the balance of things on your plate, changing the type of foods you choose (e.g. lower fat or wholegrain options), changing the way you cook food (using less fat, grilling rather than frying etc) and eating less overall are four ways to make your diet healthier.

10. Aim for a diet that is both healthy and enjoyable – only make changes that you can live with!

Drinks that are high in calories include fizzy drinks containing sugar and also fruit juices. **Recent research shows that fruit juices are as unhealthy as many fizzy drinks** and so you should not count fruit juice as one of your 'five a day' (this contradicts previous advice about fruit juice). Whole fruit however is very good for you, as it has fibre and vitamins and the sugars are not as concentrated as they are in the juice.

Recent evidence suggests that eating protein also keeps you feeling fuller for longer and so having more protein (but not more fat) could be helpful. You can get protein without fat by eating more pulses (lentils, beans). Lean meat, fish, eggs and fat-free dairy products can also be a good source of low-fat protein.

Nuts are a good source of protein and are high in fibre, so they can be a good alternative to snacks that are high in saturated fat, but they do still contain high levels of fat, so eat them in moderation.

Good sources of advice, with healthy recipes (including recipes for South Asian diets and eating healthily on a budget), are the NHS LiveWell and British Heart Foundation websites.

Keeping it off

Losing weight is one thing, but the real challenge is keeping it off! The following tips might help:

- Instead of making drastic changes that are hard to keep up, try making a series of small changes that will reduce calorie intake (e.g. less sugar in tea, low fat options, 10% less of everything). Only make changes that you can live with!
- Build up your 'resilience' by keeping track of what situations lead you to go off track with your eating. Think about ways that you can solve the problem when these situations occur.
- Weigh yourself regularly and try to figure out what makes your weight go up and down over time.
- It can be worth thinking about why you were eating more food than your body needs in the first place – what does eating that extra food do for you? For some people it is about managing stress or boredom, for others it is just going along with what other people want to do and for others it is just about the sheer pleasure of eating. The problem is that if you make some changes to your lifestyle, you will then be missing out on something (whatever the food was doing for you previously). **So, ask yourself “how else could I fulfil that need?”** – is there another way to get what you need without resorting to eating? If you can figure this out, then you may find that the 'tension' or pressure that you feel from 'being on a diet' goes away and it becomes a lot easier to manage your weight.
- Stress or boredom are often triggers for overeating, so think about other ways that you can manage stress or boredom (using the Stress Management techniques in this manual for instance).
- If you do have a lapse, don't beat yourself up about it – just make sure that you get back on track the next day.
- Support from others really helps, so if you live with someone else, have a serious conversation about how you want to change your lifestyle and how they can support you in this.

Managing Your Salt

Many people believe they eat a diet which is relatively low in salt because they don't add it at the table or when they cook. These are good habits, but you need to remember that most of the salt that we eat is in the food itself! Many processed, smoked and tinned foods are already high in salt. Foods that are often high in salt include:

- cheese
- bread (unless you make your own)
- stock cubes and stock based drinks
- tomato sauce
- soups (unless you make your own)
- cold meats
- bacon and sausages
- ready meals
- crisps and snacks

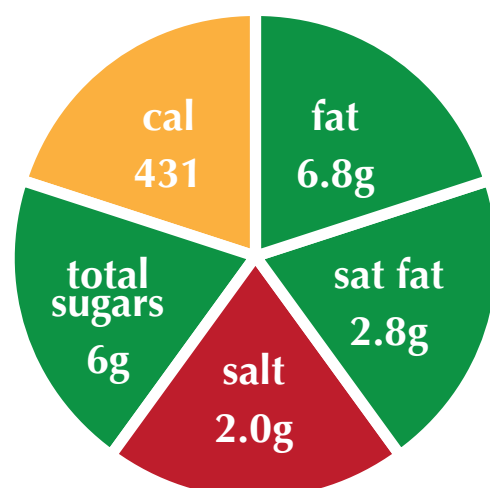
Salt is made up of two minerals, sodium and chloride. Although our bodies need these minerals they only need very small amounts. Sodium is particularly important as it is directly linked to the amount of fluid we store. The more sodium in our body the more fluid it holds. This is particularly important for people with heart failure as it can increase the pressure placed on the heart, leading to increased breathlessness and swelling.

What do the recommendations say? General healthy eating advice states that we should avoid eating more than 6 grams of salt each day. As salt is made up of both sodium and chloride this can be calculated as 2.5 grams of sodium. Knowing what foods are high, medium and low in salt is important when: shopping, preparing food and eating out.

ACTION

Go to your kitchen cupboard and take out a tin, packet or jar of food. Take a look at the nutrition label, similar to the one shown below. Now look at the sodium or salt contained. The label below shows that the food contains 0.25 grams of sodium per 100grams. **Foods which are considered high in sodium have more than 0.6grams per 100 grams.** Foods which are low in sodium contain less than 0.1grams per 100grams. If only salt is shown, a high salt level is more than 1.5 grams per 100 grams and **a low salt level is less than 0.3grams per 100 grams.** This food label therefore indicates a food which has a medium salt content. Did the food you selected have a high, medium or low amount of salt?

This pack provides...



Avoiding foods which are high in salt has become easier since the introduction of colour-coded labels. Next time you're at the supermarket, compare the salt content in your usual brands. If there is an alternative which is lower, perhaps it's time to make a change?

Reducing the salt you eat can be difficult particularly if you have become used to salty foods, but you don't need to lose flavour just because you are reducing your salt. Firstly, if you use herbs and spices instead of salt, this can keep food tasting good and may spur you on to try something new. Secondly, if you reduce the amount of salt gradually, your taste buds will soon adjust (and the same is true for sugar in tea or fat in milk) – your taste buds get used to what you give them!

Don't use "salt substitutes": Some people try to reduce the salt they eat by using salt substitutes (low-sodium table salt alternatives). However, these can have high amounts of potassium – this is another mineral which must be kept in balance when living with heart failure. It is important that you avoid these products.

ACTION

For your next few shopping trips, try to find the lowest-salt options for:

Bread, cereals, snacks, sauces and any other foods you eat regularly. Processed foods (such as ready meals, cook-in sauces) are often high in salt.

NUTRITION INFORMATION

TYPICAL VALUES	PER 100G	PER SERVING
ENERGY	241kJ/58kcal	964kJ/232kcal
PROTEIN	3.0g	12.0g
CARBOHYDRATE	6.8g	27.2g
of which sugars	2.9g	11.6g